

Workplace mental health is treated privately, but disclosure at work remains constrained.

A quantitative survey analysis of the OSMI Mental Health in Tech dataset

Survey responses

1,259

Sought treatment

50.6%

Coworker comfort

17.9%

The project tests how workplace support and stigma relate to treatment-seeking.

Research question: What workplace and personal factors are associated with mental health treatment-seeking and disclosure comfort among technology workers?

**H
1** Benefits awareness is associated with treatment-seeking.

**H
2** Expected negative consequences reduce disclosure comfort.

**H
3** Work interference is associated with treatment-seeking.

**H
4** Support indicators differ by company context.

DATASET

The Kaggle survey gives enough structure for a reproducible quantitative study.

The raw file has 1,259 responses and 27 columns; the cleaned analysis file has 56 columns after variable creation.

Raw rows

1,259

Raw columns

27

Cleaned columns

56

Invalid ages fixed

8

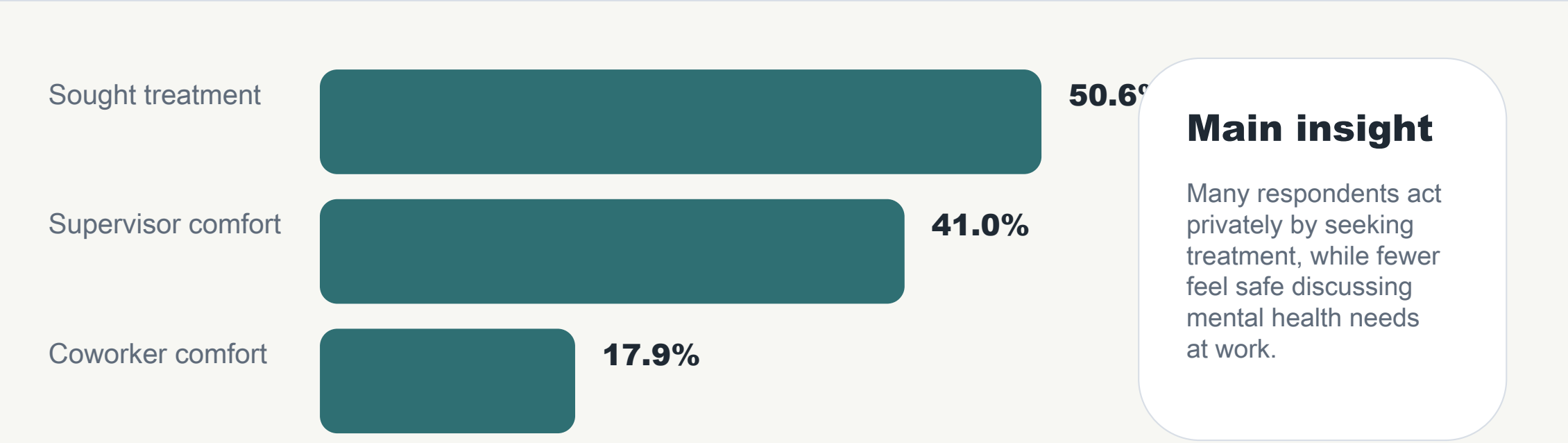
Cleaning choices

Ages outside 18-75 were treated as missing. Gender values were grouped from 49 raw responses into four broader analysis categories. Binary and ordered variables were created for treatment, benefits, work interference, leave difficulty, and disclosure comfort.

KEY PATTERN

Treatment-seeking is common, but workplace disclosure comfort is much lower.

50.6% sought treatment, while 41.0% would discuss mental health with a supervisor and only 17.9% with coworkers.



Benefit and care-option awareness are linked with higher treatment-seeking.

Treatment rate is 63.9% when benefits are known, compared with 37.0% when respondents do not know whether benefits exist.

Employer benefits

Known benefits	63.9%
No benefits	48.4%
Don't know	37.0%

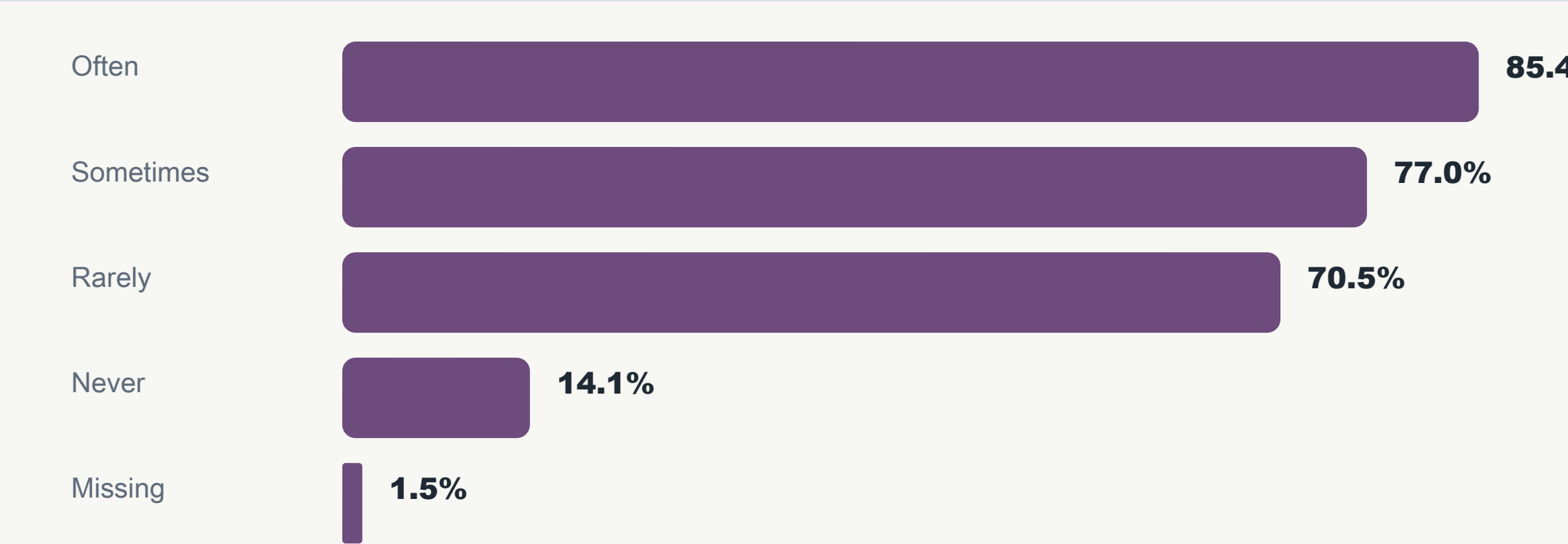
Care options

Known options	69.1%
No options	41.3%
Not sure	39.2%

WORK INTERFERENCE

Treatment-seeking rises sharply when mental health interferes with work.

85.4% of respondents reporting frequent work interference had sought treatment; only 14.1% of those reporting no interference had done so.



DISCLOSURE RISK

Expected consequences are strongly connected to lower discussion comfort.

Perceived negative consequences are statistically associated with both supervisor and coworker discussion comfort.

Supervisor consequence test

χ^2 461.66

Coworker consequence test

χ^2 285.53

Both p-values

< 0.001

Interpretation

When respondents expect negative consequences, workplace mental health conversations become less comfortable. This supports the stigma/disclosure-risk part of the capstone story.

MODEL

The treatment-seeking model separates groups well, but it remains associative.

Logistic regression predicted treatment-seeking with ROC AUC 0.891 on the test set.

ROC AUC

0.891

Accuracy

0.819

Recall

0.862

Positive model signals

Work interference, family history, coworker discussion comfort, care options, and benefits.

RECOMMENDATIONS

The practical response is to make support visible, confidential, and culturally safe.

Recommendations connect directly to the evidence: benefits, care options, disclosure comfort, and work interference.

1	Clarify benefits	Make mental health benefits easy to find and understand.
2	Protect confidentiality	Communicate anonymity and privacy protections clearly.
3	Train managers	Only 41.0% were comfortable discussing mental health with supervisors.
4	Reduce stigma	Only 17.9% were comfortable discussing mental health with coworkers.
5	Monitor interference	Use work interference as an early signal for support needs.

LIMITATIONS

The study is useful for evidence, but it should not be overclaimed.

The correct interpretation is association in a self-reported, non-random public survey sample.

- Observational data cannot prove cause and effect.
- Survey respondents are not a random sample of all technology workers.
- Self-reported answers may include recall or social desirability bias.
- Some variables have missing values, especially work_interfere.
- Gender grouping simplifies diverse self-described identities.

CLOSE

Private treatment is common, but workplace openness remains constrained.

50.6% sought treatment, but only 41.0% were comfortable with supervisors and 17.9% with coworkers.

Final takeaway

Offering mental health resources is not enough. Technology employers need visible benefits, trusted care options, confidentiality, and team cultures where mental health can be discussed safely.

Source: OSMI Mental Health in Tech Survey via Kaggle. Analysis files, notebook, dashboard, and report are saved in the project repository.